

CLINICAL INTELLIGENCE TREND REPORT

GREEN: Routine Review

Patient ID: S (Bed)Period: March 15 to April 14, 2024Report Date: April 15, 2024Coverage: 160/744h (22%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

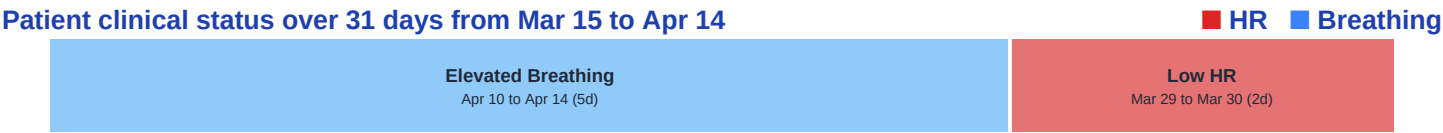
Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm



Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: GREEN

Vital	Avg	Min	Max
Heart Rate	66 bpm	52 bpm	79 bpm
Breathing	19 brpm	12 brpm	31 brpm

Limited data in the final 24h (12/24h); values reflect available readings.

Last 24 hours: one elevated breathing episode overnight, otherwise within normal range.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Apr 14	12 AM to 1 AM	1h	1	Elevated Breathing	29	25	31	Check SpO2 and lung sounds; assess for fluid overload or early infection

5 episodic events Detected, spanning 6h total. Conditions: Elevated Breathing, Low Heart Rate. Priority grade: Skip

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
5	6h	Low Heart Rate	<1	22%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Mar 29	9 PM to 11 PM	3h	2	Low Heart Rate	43	40	69	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Apr 10	8 AM to 9 AM	3h	3	Elevated Breathing	26	10	34	Check SpO2 and lung sounds; assess for fluid overload or early infection

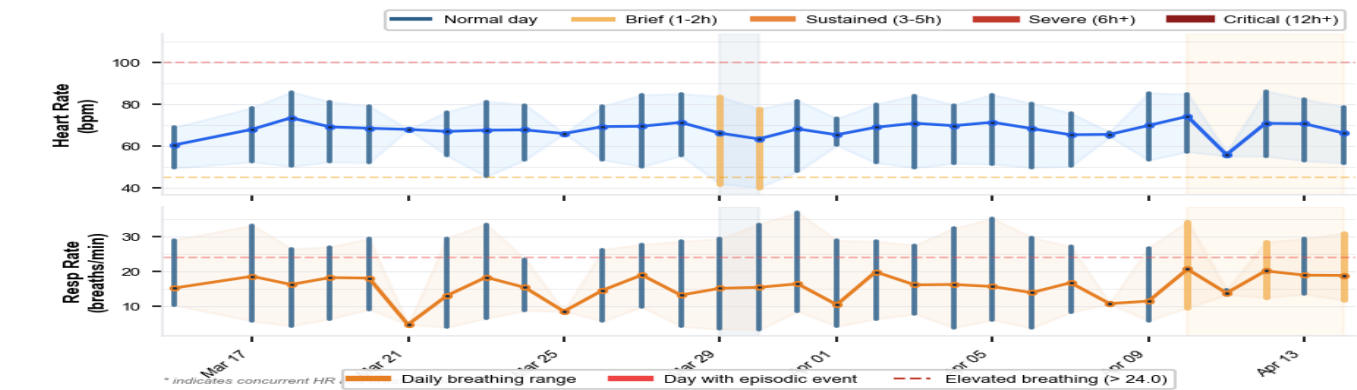
Insufficient data coverage (22%, below 30%).

Suggested Clinical Review Actions

- Elevated Breathing (avg 26 brpm, peak 34 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Low Heart Rate (avg 43 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Overall trajectory shows 2 unstable phases with 19 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

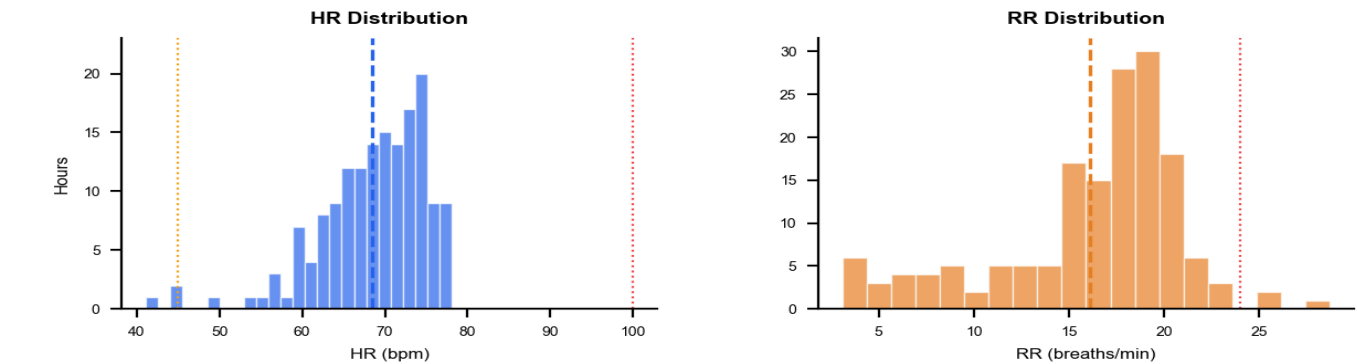
Clinical Pattern Observations:

- 1. **Clustered pattern:** Episodic events on 5 of 31 days (16%).
- 2. **Nocturnal pattern:** 4 of 5 eligible episodes during evening or night hours.

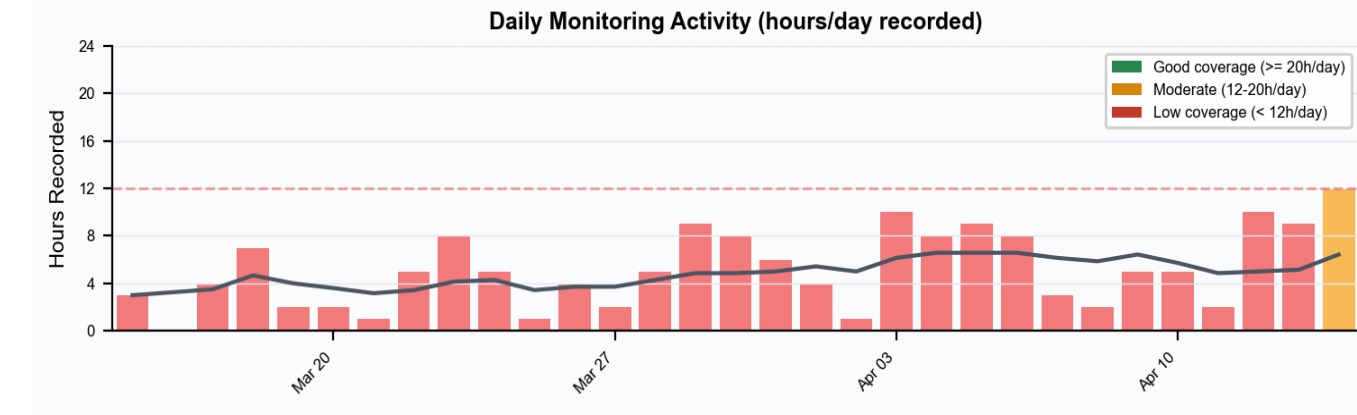


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.